

Sample #

Gender: Male

DOB:

Race: White

Report Date:

Pathology Report:

ICD-O-3 NDS
Carcinoma, urothelial
C67.3 8/20/13
Site Anterior wall of
bladder C67.3
path
Overlapping lesion
bladder C67.8
7/22/13

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Left pelvic lymph nodes; dissection:

- Eleven lymph nodes, no tumor (0/11).

B. Distal left ureter; excision:

- Ureter, positive for urothelial carcinoma in situ; no invasive carcinoma seen, see comment.

C. Right pelvic lymph nodes; dissection:

- Fourteen lymph nodes, no tumor (0/14).

D. Urinary bladder, prostate, seminal vesicles and vas deferens; cystoprostatectomy:

- Bladder with high grade urothelial carcinoma invading thru the muscularis propria and into the perivesical soft tissue, see pathologic parameters.

- Prostatic urethra with urothelial carcinoma invading into the prostatic stroma.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma.

2. Grade of tumor: High grade.

3. Depth of invasion: Extravesicular soft tissue.

4. Tumor distribution: Multifocal, 4.5 cm invasive carcinoma at right posterior, lateral and anterior wall; invasive carcinoma at prostatic urethra and prostate.

5. Ureteral margins: Left ureter, positive for in situ carcinoma; negative for invasive carcinoma; Right ureter, negative for tumor.

6. Distal urethral margin: Negative for tumor.

7. Soft tissue margin or serosa: Negative for tumor.

8. Lymph nodes: Negative for tumor (0/25).

9. pTNM: pT3a,N0,MX.

UUID: 74DA50BD-1762-42A2-84A3-637C8C1D435B
TCGA-FD-A6TK-01A-PR

Redacted



Effective January 1, 2010 this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Comment

On part B, there is pagetoid spread of urothelial carcinoma in situ involving the left ureter margin. Immunohistochemical stains performed on the distal left ureter shows that the neoplastic urothelial cells are highlighted by CK20 and p53 (nuclear) positivity. CD44 highlights the background urothelium. This immunostaining pattern confirms the above diagnosis. Controls appropriate.

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [] M.D.

Electronically Signed Out by [], MD

Clinical History:

Patient is a year-old male with bladder cancer undergoing cystectomy.

Specimens Received:

A: Left pelvic lymph nodes

B: Distal left ureter

C: Right pelvic lymph nodes

D: Bladder, prostate, seminal vesicles, vas deferens

Gross Description:

The specimens are received in four containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "left pelvic lymph node". Received fresh and placed in formalin is a 4.5 x 4.0 x 1.5 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 0.2-2.5 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

A1: 4 lymph node candidates

A2-3: 3 lymph node candidates in each cassette

A4: One lymph node candidate, bisected

B. The second container is additionally identified as, "distal left ureter". Received fresh and placed in formalin is an unoriented, 0.7 cm long segment of ureter with small amount of adipose tissue attached. The specimen is sectioned and entirely submitted as B1.

C. The third container is additionally identified as, "right pelvic lymph nodes". Received fresh and placed in formalin is a 5.0 x 4.5 x 2.7 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 0.5-3.5 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

C1: 8 lymph node candidates

C2: 4 lymph node candidates

C3: 3 lymph node candidates

C4: 2 lymph node candidates

C5-6: One lymph node candidate, bisected in each cassette

D. The fourth container is additionally identified as, "bladder, prostate, seminal vesicles vas deferens". Received fresh and placed in formalin is a 244.4 gram, 15 x 11 x 4.5 cm cystoprostatectomy specimen with attached mesenteric fat. The bladder measures 6.5 x 6 x 2.5 cm. The right ureteral stump measures 3.1 cm and the left 1.8 cm, and both demonstrate patent lumina. The prostate measures 4 x 2.8 x 2.7 cm. The right seminal vesicle measures 2.2 x 1.1 x 0.5 cm and the right vas deferens 10 x 0.5 cm. The left seminal vesicle measures 2.8 x 1.3 x 0.6 cm and the left vas deferens 9 x 0.5 cm. The right half of the prostate and bladder is inked blue and the left half is inked black. The prostate is opened anteriorly along the urethra and continued superiorly through the bladder in a Y-shaped incision. The prostate is serially sectioned from apex to base to reveal unremarkable parenchyma. The bladder demonstrates a 4.5 x 1.8 cm ulcerated area (lesion #1) involving the right posterior/lateral/anterior wall. Serial sectioning reveals tumor invasion into the muscularis propria to a depth of 0.8 cm, extending to 1.9 cm of the inked margin. There is another 1.4 x 0.7 cm ulcer in the left posterior/lateral wall (lesion #2). Serial sectioning reveals invasion into [] to a depth of [] cm, extending to [] cm of the inked margin. The remainder of the mucosa is tan-brown, edematous, and glistening with a uniform 0.7 cm wall thickness. The bilateral ureteral orifices are identified and probe patent. Representative sections are submitted as follows:

D1: Distal urethral margin

D2: Left ureter margin

D3: Right ureter margin

D4: Prostatic apical margin

D5-7: Left side of prostate

D8-10: Right side of prostate (prostate 60% submitted)

D11: Seminal vesicles and vasa deferentia

D12-13: Lesion #1, deepest invasion, tandem sections

D14: Lesion #1, deepest invasion, full thickness section

D15: Lesion #1

D16-18: Lesion #2, entirely submitted

D19: Uninvolved dome

D20: Uninvolved anterior

D21: Uninvolved trigone

Case No.	7/10/13	Yes	No
Diagnosis Discrepancy			✓
Primary Tumor Site Discrepancy			✓
IPAA Discrepancy			✓
Prior Malignancy History			✓
Dual/Synchronous Primary Note			
Case Is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initials	CMA	Date Reviewed:	7/9/13